

PRE-DEVELOPMENT APPROVAL PACK

BOOKDOC2026 Clinic Management SaaS - Delhi Pilot

Prepared: 2026-08-09

Product focus: Physiotherapy and Orthopaedics clinic management

Deployment: One centrally operated multi-tenant SaaS deployment

Document status: Baseline for stakeholder review - implementation not authorized

Decision requested: Approve this pack as the pre-development baseline and authorize the remaining discovery workshops and evidence collection only. This document does not authorize code, database migration, vendor purchase or clinical go-live.

1. Executive baseline

BOOKDOC2026 will be a centrally operated clinic-management SaaS serving independent clinic customers. Each customer is an isolated tenant with its own organizations and branches. Delhi is the first pilot jurisdiction, and Physiotherapy plus Orthopaedics are the first specialty configurations.

Dimension	Approved planning value	Status
Clinic tenants	10 in year one; 50 by year three	Confirmed
Typical clinic	1-20 staff; fewer than 10 branches	Confirmed
Normal tenant capacity	Up to 100,000 registered patients; about 100 appointments/day	Confirmed
Large owner tenant	About 20x normal data; about 2,000 appointments/day	Confirmed
Onboarding	Clinic-submitted or operator-created application; platform approval before activation	Confirmed
Branch configuration	Authorized branch administrator; no platform approval	Confirmed

2. Product and architecture decisions

- Use the SDTS.ERP2026 modular-monolith pattern with BookDoc2026 namespaces and module-owned schemas.
- Use Tenant -> Organization -> Branch as the durable commercial and security hierarchy.
- Keep platform-operator control-plane access separate from clinic Admin, Portal and patient experiences.
- Use one SQL Server database initially with mandatory tenant scope; database-per-tenant is not assumed.
- API owns commands, permissions, monitoring and administration. Production durable work runs in a separate Worker process.
- Admin owns report management and scheduling; Portal exposes permission-approved operational reports.
- Browser/PDF is normal printing. A constrained local agent may later print only receipts, queue slips and labels.

- MAUI starts with patient journeys after stable APIs; staff stages follow field validation.

3. Clinic onboarding and branch governance

1. A clinic applicant submits an onboarding form and secured evidence, or a platform operator directly creates the application.
2. The system resolves the current platform-managed document-type requirements for jurisdiction and clinic type.
3. A platform operator reviews evidence, records reasons and explicitly approves, rejects or requests resubmission.
4. Approval provisions the isolated tenant and first owner; submission alone creates no active tenant access.
5. Authorized branch admins manage allow-listed branding and numbering with scope, version and audit controls.
6. Provider/legal verification gates domains and communication identities; no BOOKDOC platform approval is required.

Control: Only platform operators may create, version, activate or retire onboarding document types. Files remain in secured document storage with validation, malware/quarantine state, hashing, access audit, expiry and replacement lineage.

4. Roles and host ownership

Role	Primary experience	Key outcome	Critical restriction
Platform operator	Control plane	Approve tenants; manage document types; monitor verification	No standing clinical access
Clinic owner/admin	Admin + permitted Portal	Configure and govern own tenant/branches	No tenant activation or platform catalog control
Receptionist	Portal	Patients, booking, arrival and queue	Masked clinical/financial fields
Doctor/Orthopaedist	Portal; MAUI later	Encounter, prescription, investigation and sign	Care relationship and branch scope
Physiotherapist	Portal; MAUI later	Assessment, care plan, sessions and discharge	Prescription authority not implied
Nurse	Portal; MAUI later	Vitals, queues and assigned tasks	Signing/prescribing separately granted
Cashier	Portal	Invoice, payment, receipt and close	Refund/discount approval separated
Radiology/Lab technician	Portal	Department worklist and status	Verification/release separately granted
Patient	Patient Portal + MAUI	Own bookings, queue, bills and released records	Self/approved representation only

5. Specialty preparation

5.1 Physiotherapy

The specialty extension covers initial assessment, pain/range-of-motion/strength and functional baselines, goals-based care-plan versions, scheduled therapy courses, session documentation, exercise programs, reassessment and discharge/referral. Package consumption must never erase clinical session evidence.

5.2 Orthopaedics

The specialty extension covers musculoskeletal history and examination, body site and laterality, red flags, imaging/investigation order and review, diagnosis, medication, clinic procedures, physiotherapy referral, external surgery/hospital referral and follow-up. A schedule or proposal never proves that a procedure occurred.

Specialty gate: Ten anonymized acceptance cases per specialty, approved templates/outcome tools, professional permissions, red-flag behavior, consent, signing/amendment, documents and billing rules are required before clinical implementation.

6. Reporting, Worker and printing

Area	Target decision	Legacy retirement evidence	Status
Backend Worker	Typed durable handlers; API-hosted development and separate production Worker	Task inventory, shadow/parallel run, retry/dead-letter/idempotency	Proposed
Reporting	HTML preview, PDF, XLSX and CSV; Admin schedules; Portal operational catalog	45 primary layouts + 43 variants classified and golden-tested	Proposed
Snapshots	Regenerate operations; retain issued clinical/financial/official outputs	Historical accessibility and retention approval	Proposed
Printing	Browser/PDF first; agent only receipt/slip/label	Offline, duplicate, wrong-branch and printer evidence	Deferred

7. India and Delhi preparation

The pack uses the DPDP Act/Rules, ABDM Health Data Management Policy, EHR Standards for India and GST invoice rules as design inputs. Delhi establishment, professional registration, prescription, biomedical-waste, fire/building and diagnostic obligations require named legal and clinic review. The software will capture configurable evidence and expiry; it will not claim to certify compliance automatically.

- MeitY DPDP Rules 2025: <https://www.meity.gov.in/documents/act-and-policies/digital-personal-data-protection-rules-2025-gDOxUjMtQWa>
- ABDM Health Data Management Policy: https://abdm.gov.in/static/media/health_management_policy_bac9429a79.80f74bc3e039c00acd4f.pdf
- Delhi Health forms/acts/rules: <https://health.delhi.gov.in/health/forms-act-rule>
- Delhi Physiotherapy Council: <https://health.delhi.gov.in/health/delhi-physiotherapy-and-occupational-therapy-council>

8. Implementation readiness

Gate	Evidence required	Current result	Next action	Status
Discovery	Live schema, anonymized sample, secret rotation	Not met	Obtain source evidence	Blocked
Security/privacy	Threat model, data-flow map, tenant isolation, Delhi review	Not met	Name reviewers	TBD
Physiotherapy	Approved templates/measures and 10 cases	Not met	Run specialty workshop	TBD
Orthopaedics	Approved exam/procedure/referral and 10 cases	Not met	Run specialty workshop	TBD
Database/migration	Profile, ownership, mappings and reconciliation	Not met	Profile live database	Blocked
Worker	Legacy task inventory and controlled replacement design	Not met	Inventory service tasks	TBD
Reports	45+43 catalog, owners and golden evidence	Not met	Run report classification	TBD
Operations	RPO/RTO, concurrency, vendors, support and recovery	Not met	Complete operational decisions	TBD
Reference slice	Tenant/branch/catalog/API/audit/Worker/isolation	Not authorized	Start after gate approval	Blocked

9. Deliverable index

The version-controlled Markdown master remains the authoritative navigator. The following working artifacts support workshops and approvals:

Artifact	Purpose	Format
BOOKDOC2026-Requirements-and-Traceability	Roles, workflows, requirements, tests, risks and decisions	XLSX
BOOKDOC2026-Migration-Data-and-Reports	Screen map, data dictionary, migration map, report and onboarding catalogs	XLSX
BOOKDOC2026-API-and-Operational-Readiness	API permissions, integrations, environments, gates and volumes	XLSX
BOOKDOC2026 Pre-development Approval Pack	Formal review and sign-off summary	DOCX + PDF
Pre-development diagrams	Architecture, onboarding and durable-work flows	Markdown/ Mermaid

10. Approval record

Approval confirms the planning baseline and permission to continue discovery and preparation. It does not authorize implementation, production data movement, vendor purchase or go-live.

Role	Name	Decision	Signature / evidence reference	Date
Product owner	TBD	Approve / revise	TBD	TBD
Clinical safety owner	TBD	Approve / revise	TBD	TBD
Finance owner	TBD	Approve / revise	TBD	TBD
Privacy/legal reviewer	TBD	Approve / revise	TBD	TBD
Architecture owner	TBD	Approve / revise	TBD	TBD
Operations owner	TBD	Approve / revise	TBD	TBD

Approval conditions

- All TBD or Blocked items remain visible and are assigned before their dependent module begins.
- No production credentials, patient records or unredacted extracts are added to the planning repository.
- Changes to scope, tenancy, specialty safety, estimates or release gates update the master, register and affected workbooks.
- Implementation begins only through the readiness gate and a separately approved reference-slice backlog.